

condition, but *because we have defined it as existing*. This doesn't mean that the virus causing measles does not exist or that it doesn't cause a fever and rash. It does mean, though, that when we talk about measles as an illness, we have organized our ideas about that virus, fever, and rash in only one of the many possible ways. In another place or time, people might conceptualize those same conditions as manifestations of witchcraft, as a healthy response to the presence of microbes, or as some other illness altogether. To sociologists, then, *illness*, like *crime* or *sin*, refers to biological, psychological, or social conditions subjectively defined as undesirable by those within a given culture who have the power to enforce such definitions.

MEDICINE AS SOCIAL CONTROL

In everyday life, we use the word *medicine* to refer to the drugs that doctors prescribe. But we can also use the word *medicine* to refer to the world and culture of doctors. For example, we might say that modern medicine is an exceedingly complex enterprise or that modern medicine primarily focuses on treating disease rather than on looking for environmental causes of illness. Even more broadly, sociologists refer to medicine as an *institution*. Sociologists use the term **institution** to refer to enduring social structures that meet basic human needs such as the religion and education. When we talk of medicine as an institution, we refer to the world and culture of doctors as well as to the economic, social, and political underpinnings of that world. We might, for example, talk about how the power of medicine *as an institution*—doctors, hospitals, the medical way of thinking about the world, and so on—has grown over the past century. In addition, sociologists increasingly talk about *biomedicine* as an institution. **Biomedicine** refers to the ways in the which medicine, science, and technology often now work together as one social institution—an institution that can increase or reduce the power of medicine as an institution (Clarke et al., 2003, 2010). (For simplicity's sake, we will primarily use the term *medicine* in this book.)

One central concept in the sociology of health and illness is the idea that medicine is, among other things, an institution of social control. **Social control** refers to the formal and informal methods used by a social group to ensure that individuals conform to social norms and to protect the existing balance of power among groups. When we say that medicine is an institution of social control, we are saying that medicine is a basic structure of our society that sometimes serves to “keep people in line.” For example, doctors have the power to decide whether someone is truly ill and deserves sympathy or is simply lazy and should be shunned. In such a situation, doctors act as **social control agents**: individuals or groups (such as parents and religious leaders) that enforce social norms. “Contemporary Issues: Citizenship and Biomedicine,” p. 103, explores the powerful rule that the institution of biomedicine now plays in deciding who can become a U.S. citizen.

In the next sections, we will see how doctors—as agents of social control—both decide which conditions or behaviors should be labeled illness and press sick people to get well and return to normal social roles.

CONTEMPORARY ISSUES

Citizenship and Biomedicine

Becoming a U.S. citizen is a long and difficult process. Each year, however, the United States allows some refugees and immigrants to bypass the usual procedures if they are the spouse, parent, or young child of a U.S. citizen. Increasingly, however, the federal government has mandated that individuals must first prove their family relationship through expensive genetic testing (Dove, 2013; Lakhani and Timmermans, 2014).

The government, of course, has both a need and an obligation to prevent immigration fraud. At the same time, relying on DNA testing leads to a different set of problems (Dove, 2013; Lakhani and Timmermans, 2014). Most basically, it enshrines biomedical information as uniquely accurate and meaningful: more important than legal documents, a parent's sworn statement, or a child's obvious desire to be reunited with the adults he or she loves. Similarly, it implicitly declares that we are all defined by our genes. In addition, it defines family in narrow terms, omitting stepchildren and adopted children. Yet in many other cultures, it is common for adults to informally adopt nieces, nephews, cousins, or others when parents die or are unable to care for them. This is especially true in the war-torn areas where most current refugees were born. Relying on genetic testing also has the effect of denying legitimacy to polygamous families in which all wives may consider all children born into the family to be their own. Finally, mandated genetic testing can rip apart—rather than unite—families when it reveals that a child is not genetically related to a man long assumed to be his or her father.

Creating Illness: Medicalization

The process through which a condition or behavior becomes defined as a medical problem requiring a medical solution is known as **medicalization** (Busfield, 2017; Conrad, 2007). For example, during the 19th century, most Americans considered chronic drunkenness to be a sin, but by the mid-20th century many instead considered it a form of mental illness. Similarly, over the last few decades various natural conditions and processes such as uncircumcised penises, male balding, aging, loss of sexual desire, and pregnancy have all increasingly come to be seen as medical problems (Armstrong, 2000; Conrad, 2007; Hartley, 2006; Rosenfeld and Faircloth, 2005). The term *medicalization* also refers to the process through which the definition of an illness is *broadened*. For example, when doctors expanded the definition of osteoporosis to include anyone with low bone density rather than only individuals who had experienced unusual bone fractures, the number of persons diagnosed with osteoporosis almost doubled (Grob and Horwitz, 2009).

For medicalization to occur, one or more organized social groups must have both a vested interest in medicalization and sufficient power to convince others (including doctors, the public, and insurance companies) to accept their new definition of the situation. It's not surprising that doctors often promote medicalization, given that it can increase their power, the scope of their practices, and their incomes. For example, during the first half of the 20th century,

improvements in the standard of living coupled with various public health measures substantially reduced the number of seriously ill children. As a result, the market for pediatricians declined, and their focus shifted from serious illnesses to minor childhood illnesses and well-baby care. Pediatrics thus became less well paid, interesting, and prestigious. To increase their market while obtaining more satisfying and prestigious work, some pediatricians have expanded their practices to include children whose behavior concerns their parents or teachers and who are now defined as having attention-deficit hyperactivity disorder (ADHD) (Conrad, 2007). Doctors have played similar roles in medicalizing crooked noses, obesity, drinking during pregnancy, impotence, and numerous other conditions (Grob and Horwitz, 2009; Loe, 2004).

In other instances, however, doctors have actively opposed medicalization (Swoboda, 2008). By definition, this is the case with any **contested illness**: distressing and painful symptoms that affected individuals believe constitute an illness even though many doctors disagree. For example, fibromyalgia is characterized by many common symptoms, including pain, dizziness, insomnia, and headache. Moreover, no blood test or x-ray can identify an individual as having fibromyalgia. As a result, many doctors question whether it really is a disease. The same is true for chronic fatigue syndrome and multiple chemical sensitivity, among others. In these cases, consumers often press for medicalization to get validation for their experiences, stimulate research on treatments and cures, and get health and disability insurance coverage for their problems (Barker, 2005, 2008; Conrad, 2007). The rise of the Internet has made it much easier for such consumers to find each other, reaffirm each other's sense that they suffer from a real illness, and lobby for medicalization.

Case Study: Working Together to Medicalize ADHD Successful medicalization often depends on the interwoven interests and activities of multiple interest groups. The history of ADHD illustrates this process.

Scientists have yet to discover any biological markers (such as viruses or genes) for ADHD (Furman, 2009). As a result, individuals are diagnosed as having ADHD or not based solely on doctors' assessment of whether the individual is too active, impulsive, or distractible (Conrad, 2007).

The diagnosis only became popular in the 1960s, following a massive advertising campaign for Ritalin (methylphenidate). That campaign aimed to convince both doctors and the public that ADHD was a real disease and that Ritalin was a safe treatment. Ritalin is indeed safer than other amphetamines and can improve individuals' concentration, impulse control, and discipline in the short term. But it may cause addiction, loss of appetite, sleep deprivation, headache, stomachache, and cancer (Davis, 2007; Vastag, 2001), and it does *not* improve individuals' chances of graduating high school, holding a job, avoiding drug abuse, or avoiding trouble with the law (Diller, 1998).

Despite these problems, pediatricians proved a ready audience for this marketing campaign, which promised a way to boost their flagging income and prestige. Teachers, too, began recommending that certain students get tested (and treated) for ADHD, in part because the drugs could make students more manageable, and the diagnosis could shift blame for student problems away from the teachers themselves (Diller, 1998).

More recently, those recommendations have turned into near requirements. Over the last 20 years, teachers and schools have faced dramatically increased pressure to maintain or increase their students' scores on standardized tests. One easy way to do so is to increase the number of students who receive Ritalin or similar drugs. As a result, teachers may press parents to get their children diagnosed and dosed. This is especially true in affluent districts where parents can afford to send their children elsewhere and schools must compete for good students (King et al., 2014). Similarly, parents may seek to have their child diagnosed in hopes that the drugs will improve their children's grades. In addition, an ADHD diagnosis gives children the benefits federally guaranteed to any children with disabilities, including special educational services and protection against suspension for disciplinary problems related to ADHD (Conrad, 2007).

Around 10% of U.S. schoolchildren—and almost 20% of high school boys—have been diagnosed with ADHD (Schwarz and Cohen, 2013). In addition, diagnoses have spiked among toddlers, even though official definitions of ADHD limit it to children age four and older (Schwarz, 2014).

Unintended Consequences of Medicalization In some circumstances, medicalization can be a boon, leading to social awareness of a problem, sympathy toward those diagnosed with an illness, and the development of helpful treatments. Persons with epilepsy, for example, lead far happier and more productive lives now that their seizures are treated with drugs rather than treated as signs of demonic possession. But defining a condition as an illness does not necessarily improve the social status of those who have that condition. Those who use alcohol excessively,

for example, continue to experience social rejection even when alcoholism is labeled a disease. Moreover, medicalization also can lead to new problems, which sociologists refer to as **unintended negative consequences** (Conrad, 2007).

First, once a situation becomes medicalized, doctors become the only experts considered appropriate for diagnosing the problem and defining appropriate responses to it. As a result, the power of doctors increases while the power of other social authorities (including judges, the police, religious leaders, legislators, and teachers) diminishes. For example, now that troublesome behavior by children is increasingly diagnosed as ADHD, parents, teachers, and the children themselves have lost credibility when they disagree with this diagnosis. Similarly, doctors are now given considerable authority to answer questions such as who should receive abortions or organ transplants, how society should respond to drug use, and whether severely disabled infants should receive experimental surgeries, while the authority of religious leaders and family members to answer these questions has diminished.

As this suggests, medicalization significantly expands the range of life experiences under medical control. For example, the natural process of aging is increasingly regarded as a medical condition. Doctors now scrutinize all aspects of the aging body and recommend psychological tests to measure mental decline, hormones to improve virility, cosmetic surgery for wrinkles, and more (Conrad, 2007).

Second, once a condition is medicalized, medical treatment may seem the only logical response to it. For example, if woman battering is considered a medical condition, then doctors need to treat women and the men who batter them. However, if woman battering is considered a social problem stemming from male power and female subordination, then it makes more sense to arrest the men, assist the women financially and emotionally, and work for broader structural changes to improve all women's status and options.

Third, when doctors define situations in medical terms, they reduce the chances that these situations will be understood in *political* terms. For example, China, Pakistan, and other countries have removed political dissidents from the public eye by committing them to mental hospitals. By so doing, these governments discredited and silenced individuals who might otherwise have offered powerful dissenting voices. In other words, medicalization allowed these governments to **depoliticize** the situation: to define it as a medical rather than political problem.

Fourth, and as the examples of China and Pakistan illustrate, medicalization can justify involuntary treatment. Yet treatment sometimes harms more than it helps. For example, since the 1980s, U.S. doctors have legally forced small numbers of women to submit to cesarean deliveries, in which babies are surgically removed from their mothers' uteruses rather than delivered naturally through the vagina (Paltrow and Flavin, 2013; Roth, 2003). In these cases, doctors argued successfully that childbirth is a dangerous medical condition rather than a natural process, that doctors are better qualified than pregnant women to judge fetuses' needs, and that fetuses' right to health is more important than women's right to control their own bodies. Yet the rate of cesarean section in the United States is twice that

recommended by the World Health Organization, suggesting that doctors are far too ready to perform this potentially life-threatening surgery. "Ethical Debate: Medical Social Control and Fetal Rights," p. 108, explores how the growing acceptance of the idea of "fetal rights" is affecting the lives of pregnant women.

Medicalization and the "Potentially Ill" In addition to creating new illnesses, medicalization has also led to labeling increasing numbers of individuals as "potentially ill" (Boyer and Lutfey, 2010; Conrad, 2005; Scott, Wood, and Gray, 2005). The **potentially ill** are individuals identified as having an above-average risk of illness, whether because of age, stress level, tobacco use, family history, medical test results, or other factors.

The risks faced by the potentially ill vary substantially. Some learn that they carry a gene guaranteed to cause a fatal disease. Many more, however, learn that they have a condition such as high cholesterol that may increase their risk of illness. The numbers of such individuals continues to increase as corporations develop more tests for risk factors and as doctors (often reimbursed per test) adopt such tests. Similarly, the ranks of the potentially ill have expanded as pharmaceutical companies have encouraged both doctors and consumers to expand their ideas about health risks and to adopt treatments for those risks. For example, pharmaceutical companies have worked not only to broaden the definition of osteoporosis but also to create a new category, osteopenia, for those at *risk* of osteoporosis. Because *osteoporosis* refers to the risk of bone fractures caused by low bone density, osteopenia is essentially the *risk* of a *risk* of a health problem.

As this suggests, the health benefits of learning that one is potentially ill depend on the magnitude of the identified risk and the effectiveness of available treatments (Scott et al., 2005). Those benefits, however, must also be balanced against the psychological distress caused when people without any symptoms learn that illness might strike at any moment (Marteau and Richards, 1996). In addition, some of these individuals experience the stigma of illness without any of the benefits that those who have illnesses may receive such as legal protection from discrimination.